

The Night Shift

Three nights in an A&E; department. What happened could not have been predicted. What was learned could not have been untaught.

CEFR Level B1

Book 7 of 9

Adult Graded Reader

Setting: An A&E; department in a regional hospital in northern England. Three consecutive night shifts.

Genre: Medical Drama / Ensemble

Characters

- Dr. Amara — 32. Registrar. Her first month as a registrar, first time leading a shift.
- Nurse Kezia — 48. Senior nurse. Twenty-two years in A&E.; Calm, specific, truthful.
- Patient Tom — 66. Retired teacher. Recurring presence across all three nights.
- Dr. Holt — 55. Consultant. Present by phone, then in person.

The handover from the late shift took eleven minutes.

Amara had been listening to handovers for three years as an SHO, but this was the first time she was receiving one as the registrar in charge.

The departing registrar — Callum, who was younger than her by two years and had been doing this for fourteen months — gave her the list in the particular order A&E; registrars used: majors first, minors second, anything that might deteriorate overnight flagged in red.

'Three to watch,' Callum said. 'Bay four, bay seven, and the curtained area next to the nurses' station. Bay seven is Holt's — he'll call in at midnight.'

Amara wrote everything down.

'You'll be fine,' Callum said, which was what people said when they weren't sure.

majors majör vakalar "You'll be fine." The thing people say when they're not sure. Night one begins.

Bay seven was Tom.

He had come in at seventeen thirty with chest pain and shortness of breath. He was sixty-six, retired, had a history of hypertension that was, his GP records showed, adequately managed.

The chest pain had been ruled not cardiac after two sets of ECGs and a troponin at four hours. He was waiting for a repeat troponin at eight hours and a cardiology review in the morning.

Amara checked his observations. His blood pressure was slightly elevated. His respiratory rate was normal. He looked tired but not distressed.

'Has anyone explained what's happening?' she asked.

'The other doctor did. I'm waiting for another blood test and then someone to look at my heart.' He paused. 'I used to be a biology teacher. I understand the words.'

cardiac kalple ilgili observations vital bulgular He used to be a biology teacher. He understands the words. This will matter later.

Between twenty-two hundred and two in the morning, A&E; did what A&E; always did on a weeknight: it filled. A road traffic accident brought in three patients, one of whom required the trauma team.

A woman in her eighties with a fall and a suspected hip fracture.

Two alcohol-related presentations that were, in Kezia's phrase, 'routine complexity.'

Amara moved between bays with the focused efficiency that the job required.

Kezia moved with her — not following, but parallel — anticipating what was needed before Amara had finished requesting it.

'You've done this before,' Amara said at one point.

'Twenty-two years,' Kezia said. 'You get a feel for the rhythm.'

anticipating öngörmek "You get a feel for the rhythm."

Kezia has twenty-two years. Amara has one month.

lights dimmed, the monitors speaking softly.

'Can't sleep,' he said. 'I never can in hospitals. Too much happening.'

Amara sat down. She had been moving for six hours.

'Are you worried about the results?' she asked.

'A bit,' Tom said. 'Not as much as you'd think. I've been in enough of these situations to know that worrying doesn't help the wait.' He looked at her. 'Are you all right?'

She had not expected the question. She had been asked that question by patients before, but not at two in the morning when she was the one who was supposed to be asking it.

'I'm fine,' she said, which was what you said.

Tom looked at her with the expression of someone who had spent thirty years in front of teenagers and was not going to be persuaded by that answer.

'First shift as the lead?' 'First month,' she said. 'Tonight's my fourth shift as registrar.' 'How's it going?' 'Fine so far. Busy, but manageable.'

'That's different from answering the question,' Tom said. She looked at him. Then she said: 'It's harder than I expected. Not the medicine — I know the medicine. It's the responsibility. Being the one who decides.'

'Yes,' Tom said. 'I remember that feeling.' 'From teaching?' 'Thirty years. You're always the one who decides. You never entirely get used to it. You just get faster at trusting your own judgement.'

Tom's repeat troponin came back at six in the morning. It was negative. His chest pain was most likely musculoskeletal — he had been moving furniture the previous weekend.

Amara told him. He received the news with the calm of someone who had been prepared for either outcome.

'Will I need to see cardiology?' 'The registrar on in the morning will discuss it with them. Given your history, they'll probably want you to have a follow-up.'

'That's sensible,' Tom said.

He thanked her as she was leaving. 'Get some sleep,' he said. She had been on shift for fourteen hours.

'You too,' she said. Then: 'Thank you. For the two o'clock conversation.' 'It was mutual,' Tom said.

negative negatif (test sonucu) "It was mutual." She helped him. He helped her. Two in the morning made this possible.

Night two was different.

Amara arrived to a handover that included a flagged patient in bay three — a forty-two-year-old woman who had been brought in by ambulance with severe abdominal pain. The initial assessment had suggested several possibilities. The working diagnosis was acute appendicitis, awaiting CT confirmation.

The Night Shift



Amara starts the night shift under pressure, noise, and fluorescent light.

But the CT had been delayed — the scanner had been occupied since early evening.

By the time Amara received the handover, the patient had been waiting for four hours.

Amara went to bay three immediately.

The woman — her name was Priya — was in significant pain and had become, the departing registrar noted, increasingly anxious.

had risen in the last two hours. Her abdomen was rigid and she was guarding.

'I'm going to call surgery,' Amara told Kezia.

'The CT?' 'The CT is taking too long. She needs a surgical assessment now.'

Kezia looked at the observations. 'Agreed,' she said. One word, no elaboration.

Amara called the surgical registrar, who was, as surgical registrars often were at midnight, in theatre.

She gave the handover clearly: female, forty-two, four hours of severe abdominal pain, temperature rising, peritoneal signs.

'I'll be there in twenty,' the surgical registrar said.

Amara went back to Priya and explained what was happening.

Priya was taken to theatre at one-thirty in the morning.

The surgical team found a perforated appendix.

Amara received the update at four in the morning, when the night was quietest.

Kezia had brought two cups of coffee from somewhere.

'How did you know?' Kezia asked.

'The temperature trajectory,' Amara said. 'And the way she was guarding. Those two together, with the delay — waiting for the CT would have been the wrong call.'

'You could have waited,' Kezia said. 'Most registrars would have.'

'I know,' Amara said. 'That's why I didn't.'

Kezia looked at her coffee. 'Good,' she said.

most registrars çoğu asistan "That's why I didn't." She knew she was deviating from the usual. She did it anyway. He did not come in often — he was the overnight consultant and normally managed by phone.

But there had been a particular run of complex presentations and he had decided, he said, to come and see the department.

He arrived at midnight and spent an hour reviewing cases with Amara.

He was, she had been told, not always easy to work with — he was precise about what he expected and direct when expectations were not met.

He said, going through the previous night's case notes: 'The appendix. Good call.'

'Thank you,' she said.

'You could have waited for the CT and been right,' he said.

'But you would have been right for the wrong reason. You made the right call for the right reason. That's different.'

case notes vaka notları for the wrong reason yanlış nedenle complex presentations karmaşık vakalar "Right for the right reason. That's different." Holt draws a distinction she will not forget.

He had been discharged the morning after his first admission and had been seen by cardiology, who had arranged for further investigation.

He had come back in at twenty-two hundred — not with chest pain this time, but with his wife, who had fallen at home and possibly fractured her wrist.

He came to find Amara while his wife was being assessed.

'I remembered you were on nights,' he said. 'I thought I'd say hello. And thank you. For the conversation.'

'How are you?' she asked.

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At a patient's bedside, experience and instinct have to work together.

'Better,' he said. 'Cardiology think it was a cardiac event, but minor. I'm on medication. I'm eating differently.' He paused. 'You look less like someone who doesn't trust their own judgement.'

minor minör, küçük "You look less like someone who doesn't trust their own judgement." He has been watching. She has changed.

At three in the morning, during the quietest stretch of the third night, Kezia and Amara were in the bay next to the nurses' station.

Kezia was updating charts. Amara was reviewing the morning handover list.

'Can I ask you something?' Amara said.

'Yes.'

'When did you stop being afraid?'

Kezia put down her pen. She thought for a moment.

'I don't think I stopped,' she said. 'I think I got better at deciding what to do with it. Fear is good information. The problem is when it stops being information and starts being noise.'

'How do you tell the difference?' 'The same way you told it with the appendix case. You ask: is this fear telling me something about the patient, or is it telling me something about me?'

stopped being... olmaktan çıkmak "Fear is good information — until it becomes noise." Kezia's twenty-two years in one sentence.

The handover on the morning of the fourth day took thirteen minutes.

Amara gave it to the incoming registrar — a man she had trained alongside, who had been a registrar for six months. She went through the list in the order she had received it: majors first, minors second, anything that might deteriorate flagged in red.

She noted Priya's outcome. She noted Tom's wife's wrist, which had been confirmed fractured and was in a cast.

She noted two presentations from the last hour that would need watching.

'Anything to worry about?' the incoming registrar asked.

'Bay four,' she said. 'The temperature pattern is concerning. Call the surgical team if it goes higher than thirty-eight point five.'

'Got it,' he said. 'How were the nights?' 'Educational,' she said.

trained alongside birlikte eğitim almış cast alçı concerning endişe verici temperature pattern ateş düzeni educational öğretici "Educational." Three nights. Two difficult decisions. Two important conversations. All of it true.

The first was that the right decision was not always the same as the expected decision, and that the difference between them was what judgement was for.

The second was that fear was information until it became noise, and that distinguishing between them was a skill — not a gift, not a personality trait, but a skill you built with practice.

The third was that Tom, at two in the morning, had asked if she was all right because he was the kind of person who asked questions like that, and that she should also be the kind of person who asked questions like that.

She wrote all three in her notebook on the bus home.

She thought: these are the things that can't be taught from a textbook.

built with practice pratikle inşa edilen gift yetenek, hediye
Three things. Written on the bus home. The things that can't be taught from a textbook.

She had asked Kezia, in the third night's quiet hour, what kept her in A&E; after twenty-two years.

Kezia had thought about this properly.

'It's the same reason I started,' she said. 'In A&E, you have to respond to what's actually in front of you. There's no protocol for the specific patient at the specific moment. The protocol gets you to the edge of the decision. After that, you're on your own.'

'Doesn't that exhaust you?' Amara asked.

'No,' Kezia said. 'It's the part that keeps me awake, in the good sense. The part where I know I'm actually doing something, not just following a process.'

She paused. 'Ask me again in ten years,' she said. 'But probably still no.'

times since he had said it.

'Right for the right reason' was a higher standard than simply being right.

She had been taught, in medical school, that outcomes were what mattered. A patient who was treated correctly and recovered was a good outcome. A patient who was treated incorrectly but recovered anyway was still a good outcome.

But Dr. Holt had been saying something different: that the quality of the decision mattered independently of the outcome.

This was a harder standard. It required her to know not just what she had done, but why.

She thought: I am going to spend the rest of my career trying to meet that standard.

This was one of the more commonly missed wrist fractures because it did not always show on initial X-ray.

Amara had suspected it from the mechanism of injury and the location of the pain, and had requested a CT rather than relying on the X-ray.

Tom, who was waiting in the relatives' bay, was told when Amara came out.

'You asked for more tests than the other doctor mentioned,' he said.

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Morning comes quietly, carrying exhaustion, relief, and hard-earned trust.

'The fracture doesn't always show on the standard view,' Amara said. 'I wanted to be sure.'

Tom looked at her. 'You're getting faster at trusting your own judgement,' he said.

She thought about two in the morning, six nights ago. 'Yes,' she said. 'I think I am.'

relatives' bay hasta yakınları alanı Two in the morning, six nights ago. "You're getting faster." He has seen the change.

They were not the same as the three she had just completed — different patients, different presentations, no Tom.

But she moved through them differently.

She was still afraid, in the appropriate places. She still called for help when she needed it.

But the fear was more often information now. Less often noise.

She asked patients if they were all right, and she meant it when she asked.

She wrote in her notebook after each shift.

She was aware that she was, in some sense, being taught. Not by the shifts — by paying attention to what the shifts contained.

She ran into Kezia in the car park three weeks after the three night shifts.

They had worked together several times since but had not been on the same shift.

Kezia was finishing a day shift. Amara was starting a night.

'How's it going?' Kezia asked.

'Better,' Amara said. 'I'm still finding the right-reason part difficult. But I'm asking the question more.'

'That's progress,' Kezia said. 'The question is more than half of it.'

Amara thought about this on her way in.

She thought: Kezia has been in this department for twenty-two years and she is still asking the question. This was, she found, more encouraging than reassuring. She preferred it that way.

finding difficult güç bulmak asking the question soruyu sormak progress ilerleme "More encouraging than reassuring." She prefers it that way. That is the right preference.

By her fifty-second night shift as registrar, she had stopped counting.

She had noticed this one Wednesday morning when she was updating the log she had kept since her first night. She had written the date, then looked at the shift number and realised she didn't know it.

She checked. Fifty-two.

She thought about Tom. She thought about Priya's appendix. She thought about Kezia's definition of fear.

She had not forgotten any of it. She had incorporated it — it had become part of the way she moved through the department, rather than a lesson she was trying to apply. She thought: this is what Callum meant when he said you'd be fine.

He was right. He just didn't know yet.

part of the way she moved hareket etme biçiminin parçası updating the log kaydı güncellemek "He was right. He just didn't know yet." She became fine by learning these things. That is the story.

Story Vocabulary

The words below are collected from the story so the reading pages remain in a natural storybook format.

1. handover	devir teslim
2. registrar	uzman doktor adayı
3. SHO	stajyer doktor
4. deteriorate	kötüleşmek
5. flagged	işaretlenmiş
6. chest pain	göğüs ağrısı
7. shortness of breath	nefes darlığı
8. hypertension	hipertansiyon
9. troponin	troponin testi
10. road traffic accident	trafik kazası
11. trauma team	travma ekibi
12. suspected	şüpheli
13. routine complexity	rutin karmaşıklık
14. parallel	paralel
15. dimmed	karartılmış
16. softly	sessizce
17. overhead lights	tavan ışıkları
18. worried	endişeli
19. supposed to be asking	sorması gereken
20. situations	durumlar
21. persuaded	ikna etmek
22. manageable	yönetilebilir
23. responsibility	sorumluluk
24. judgement	yargı, karar
25. entirely	tamamen
26. lead	baş, lider
27. musculoskeletal	kas-iskelet
28. outcome	sonuç
29. follow-up	takip randevusu
30. sensible	akıllıca
31. mutual	karşılıklı
32. abdominal pain	karın ağrısı
33. acute appendicitis	akut apandisit
34. CT scan	bilgisayarlı tomografi
35. delayed	gecikmiş
36. assessment	değerlendirme
37. confirmation	doğrulama
38. rigid abdomen	sert karın

39. guarding	koruyucu kasılma
40. peritoneal signs	periton belirtileri
41. surgical registrar	cerrahi asistan
42. elaboration	açıklama, ekleme
43. in theatre	ameliyatta
44. perforated appendix	delinmiş apandis
45. trajectory	seyir, gidiş yolu
46. update	güncelleme
47. temperature trajectory	ateş seyri
48. the wrong call	yanlış karar
49. overnight consultant	nöbet konsültanı
50. precise about	... konusunda kesin
51. direct	doğrudan
52. discharged	taburcu edildi
53. admission	yatış
54. fractured wrist	kırık bilek
55. assessed	değerlendirildi
56. cardiac event	kalp olayı
57. updating charts	kayıtları güncellemek
58. noise	parazit, gürültü
59. information	bilgi
60. stretch	süre, dönem
61. tell the difference	farkını anlamak
62. incoming registrar	gelen asistan
63. personality trait	kişilik özelliği
64. distinguishing	ayırt etmek
65. textbook	ders kitabı
66. judgement	yargı, değerlendirme
67. protocol	protokol
68. specific patient	belirli hasta
69. exhaust	bitirmek, tüketmek
70. awake (figurative)	uyanık, hayatta
71. process	süreç
72. on your own	kendi başına
73. independently of	... den bağımsız olarak
74. standard	standart
75. quality of the decision	kararın kalitesi
76. incorrectly	yanlış olarak

77. treated correctly	doğru tedavi edildi
78. outcomes	sonuçlar
79. scaphoid	skafoid kemiği
80. mechanism of injury	yaralanma mekanizması
81. missed fracture	gözden kaçan kırık
82. standard view	standart görüntü
83. relying on	... e güvenmek
84. appropriate places	uygun yerlerde
85. in some sense	bir anlamda
86. paying attention to	dikkat etmek
87. contained	içerdiği şey
88. meant it	bunu kastetti
89. she moved through them	geçti onların içinden
90. ran into	rastladı
91. reassuring	rahatlatıcı
92. encouraging	cesaretlendirici
93. incorporated	içselleştirdi
94. counting	saymak
95. definition	tanım
96. apply a lesson	bir dersi uygulamak

After-Reading Activities

1. Vocabulary Practice — Match the Words

Match each word or phrase with the correct meaning.

Word / phrase

Meaning

1. handover

A. devir teslim

2. deteriorate

B. kötüleşmek

3. trajectory

C. seyir

4. peritoneal signs

D. periton belirtileri

5. scaphoid

E. skafoid kemiği

6. protocol

F. protokol

7. incorporated

G. içselleştirdi

8. outcome

H. sonuç

2. Fill in the Blanks

Choose the correct word or phrase from the box.

fear — right — judgement — information — educational —
noise

1. Amara described the three nights in one word:

_____.

2. Kezia said fear was good _____ until it became noise.

3. The problem, Kezia said, was when fear stopped being information and started being _____.

4. Dr. Holt said Amara had made the _____ decision for the right reason — and that this was different from being right for the wrong reason.

5. Tom said you never entirely get used to responsibility — you just get faster at trusting your own _____.

6. By shift fifty-two, the lessons had become part of how she worked — she had _____ them.

3. Comprehension Questions

True or False? Write T or F.

1. This is Amara's first ever night shift as a doctor. ____

2. Tom is a retired doctor. ____

3. Amara waits for the CT scan before calling surgery for Priya. ____

4. Dr. Holt comes in on the first night. ____

5. Tom's wife has a broken ankle. ____

6. By shift fifty-two, Amara has stopped counting her shifts.

Short Answer Questions

7. What does Tom say at two in the morning that helps Amara?

8. Why does Amara call the surgical team without waiting for the CT?

9. What distinction does Dr. Holt draw about Amara's decision?

10. What does Kezia say about fear?

11. What three things does Amara write in her notebook on the bus home?

12. What does Amara do differently for Tom's wife's wrist, and why?

13. What keeps Kezia in A&E; after twenty-two years?

14. What does it mean that Amara prefers to find Kezia's approach 'encouraging rather than reassuring'?

15. What does Amara mean when she says Dr. Holt's standard is 'harder'?

Answer Key

(Alıştırmaları yaptıktan sonra bakınız.)

Match: 1-A 2-B 3-C 4-D 5-E 6-F 7-G 8-H

Fill in the Blanks: 1. educational 2. information 3. noise 4. right 5. judgement 6. incorporated

True / False: 1. F 2. F 3. F 4. F 5. F 6. T

7. He tells her you never entirely get used to being the one who decides — you just get faster at trusting your own judgement.

8. The CT was taking too long, the patient's temperature was rising, and she was showing peritoneal signs — waiting would have been the wrong call.

9. He says she could have waited for the CT and been right, but would have been right for the wrong reason — she was right for the right reason, which is different.

10. Fear is good information until it becomes noise — the skill is distinguishing whether it's telling you something about the patient or about yourself.

11. 1) Right decisions differ from expected decisions; 2) Fear is information until it becomes noise; 3) She should be the kind of person who asks 'are you all right?'

12. She requests a CT rather than relying on X-ray, because scaphoid fractures don't always show on standard view.

13. In A&E; you have to respond to what's actually in front of you — the protocol gets you to the edge of the decision, but then you're on your own, and that keeps her engaged.

14. Reassuring would mean the work gets easier — encouraging means it stays demanding but she is getting better at meeting the demand. She prefers being kept alert.

15. It requires her to know not just what she decided, but why — the quality of the reasoning matters independently of whether the outcome was good.